

SM PHARMACEUTICALS SDN BHD

**TYMIDIN 200mg Tablet
and
TYMIDIN TABLETS 400MG**

DESCRIPTION:

For 200 mg tablet:

Blue colour, round shaped with standard concave surface, film-coated tablet.

For 400 mg tablet:

Blue colour, capsule shaped with deep concave surface and break line on one side, film-coated tablet.

COMPOSITION:

Each tablet contains:

For 200 mg tablet = Cimetidine 200 mg.

For 400 mg tablet = Cimetidine 400 mg.

Pharmacodynamics:

Inhibition of basal and nocturnal gastric acid secretion by competitive inhibition of the action of histamine at the histamine H₂- receptors of the parietal cells; also inhibits gastric acid secretion stimulated by food, betazole, pentagastrin, caffeine, and insulin.

Inhibits hepatic cytochrome P-450 and P-448 mixed function oxidase systems; antagonizes dihydrotestosterone (antiandrogenic action):

PHARMACOKINETICS:

Absorption:

Rapidly and well absorbed (approximately 60 to 70%) from gastrointestinal tract.

Rate of absorption, but not extent, is delayed by food.

Protein binding: Low.

Metabolism: Hepatic (30 to 40% of an oral dose).

Half-life:

Normal: 2 to 3 hours.

Creatinine clearance 20 to 50 mL per minute: 2.9 hours.

Creatinine clearance less than 20 mL per minute: 3.7 hours.

Time to peak concentrations: 45 to 90 minutes.

Duration of action: Provides up to 80% inhibition of basal gastric acid secretion for 4 to 5 hours after a 300 mg dose.

Excretion:

Primarily renal (approximately 48% of an oral dose excreted unchanged in 24 hours). Faecal (10% of an oral dose).

Excreted also in breast milk.

INDICATIONS:

Indicated in the short term treatment of active benign gastric and duodenal ulcer. Indicated in the treatment of pathological gastric hypersecretion associated with Zollinger – Ellison syndrome. Indicated in the treatment of gastro-esophageal reflux disease. Cimetidine is not recommended for a minor digestive complaint.

RECOMMENDED DOSAGE, DOSAGE SCHEDULE AND ROUTE OF ADMINISTRATION:**Usual adult dose:**

Duodenal ulcer – 400 mg of cimetidine two times a day in the morning and at bedtime, or 800 mg at night; for 4 – 8 weeks. Prophylaxis of recurrent duodenal ulcer: The equivalent of 400 mg of cimetidine at bedtime.

Gastric ulcer, benign, active – 400 mg of cimetidine two times a day in the morning and at bedtime, or 800 mg at night; for 4 – 8 weeks.

Pathological hypersecretory conditions (e.g., Zollinger-Ellison syndrome, systemic mastocytosis, multiple endocrine adenomas) – 400 mg of cimetidine two times a day in the morning and at bedtime. The dosage being adjusted as needed and therapy continued for as long as clinically indicated.

Note: For patients with impaired renal function – Cimetidine 400mg every twelve hours, the dosage being increased to 400 mg every eight hours more frequently if necessary. Further reduction in dosage may be required if hepatic function impairment is also present.

Geriatric patients with reduced renal and / or hepatic function may be more sensitive to the effects of the usual adult dose.

Usual adult prescribing limits: Up to 2.4 grams of cimetidine daily however doses up to 10 grams per day have been used in the treatment of pathological conditions.

Usual paediatric dose: 5 to 10 mg of cimetidine per kg of body weight four times a day, with meals and at bedtime.

Note: Clinical experience with the use of cimetidine in children up to 16 years of age is limited, risk-benefit must be considered.

Route of administration: Oral

CONTRAINDICATIONS:

Patients who are known to have hypersensitivity to the drug.

PRECAUTIONS / WARNINGS:**Cross-sensitivity**

Patients intolerant of ranitidine may be intolerant of this medicine also.

Tumourigenicity

Long term studies in rats have produced a statistically significant higher incidence of benign Leydig cell tumours in cimetidine treated groups than in controls.

Pregnancy / Reproduction

Although studies in humans have not been done, risk-benefit must be considered since animal studies have shown that cimetidine crosses the placenta. Also, a study in rats exposed to cimetidine during intrauterine life and immediate neonatal period showed a hypoandrogenization in adult life with decreased weights of androgen dependent tissues and decreased concentrations of testosterone.

Breast-feeding

Problems in humans have not been documented; however risk-benefit must be considered since cimetidine is excreted in breast milk.

Geriatrics

In geriatric patients with reduced renal and / or hepatic function the clearance of cimetidine may be decreased, resulting in elevated blood concentrations. Thus, these patients may be more susceptible to the toxic effects of cimetidine, especially dizziness and confusion.

INTERACTIONS WITH OTHER MEDICAMENTS:

Cimetidine apparently, through an effect on certain microsomal enzyme system, has on occasion, caused clinically significant changes in the metabolism of some drugs by delaying their elimination, therefore increasing or prolonging blood concentration of these drugs. The most important of these drugs are warfarin type anticoagulants, phenytoin and theophylline.

In case of warfarin anticoagulants, close monitoring of prothrombin time is recommended and adjustment of the anticoagulant dose may be necessary, when cimetidine is administered concomitantly.

In case of phenytoin or theophylline, dosage may require adjustment when starting or stopping concomitantly administered cimetidine to maintain safe optimum therapeutic levels.

Absorption of cimetidine is not significantly impaired by food or by concomitant administration of antacids at the usual recommended doses

PREGNANCY AND LACTATION:

Use in pregnancy and lactation – risk benefit must be considered in pregnancy and lactation since studies have shown that cimetidine crosses the placenta and is excreted in breast milk.

Before giving cimetidine to patients with gastric ulcers, the possibility of malignancy should be excluded since cimetidine may mask symptoms and delay diagnosis.

SIDE EFFECTS :

Mild and transient diarrhoea, tiredness and dizziness.

Skin rashes, sometimes severe.

Gynaecomastia and impotence have also occasionally occurred in men receiving relatively high doses for conditions such as the Zollinger Ellison syndrome. Reversible confusional states especially in the elderly or in seriously ill patients such as those with renal failure have occasionally occurred.

SYMPTOMS AND TREATMENT FOR OVERDOSE :

In the treatment of overdose, the usual measures to remove unabsorbed material from the gastrointestinal tract, clinical monitoring, and supportive therapy are recommended. In the event of respiratory failure, artificial respiration should be started and maintained.

Administration of a beta-blocker is recommended to control the tachycardia.

Effects on Ability to Drive and Use Machine:

Unknown

STORAGE CONDITIONS AND INSTRUCTIONS TO USE:

Store below 30°C, in a tight, light-resistant container.

PACKING / PACK SIZES:

For 200 mg: Blister pack (Aluminium and PVC Foil) of 10's, 10 x 10's/Box and 100 x 10's/Box.

For 400 mg: Blister pack (Aluminium and PVC Foil) of 100 x 10's/Box.

SHELF LIFE: 3 Years

Manufacturer and Product Registration Holder

SM PHARMACEUTICALS SDN BHD (218620-M)

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